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Intro to Gender, Race, and Sexuality

Who Gets to Survive? Black Maternal Mortality and Feminism

INTRODUCTION

A woman is pregnant. She is bleeding. She is in pain. Not in her back, which is where nurses keep asking about, but somewhere else. She describes it as best she can. She is educated. She has health insurance. She is married. Every credential that mainstream feminist politics tells us should protect a woman...she has *all* of them. Yet nobody listens. She goes home. She comes back three days later. The ultrasound shows tumors the size of the baby. She has been in labor the whole time. Her daughter is born barely breathing and dies within minutes. And the nurse's parting words are that there was nothing they could have done, because, she says, the patient never told them she was in labor.

That woman is sociologist Tressie McMillan Cottom. And she wrote about it in an essay called "Dying to Be Competent." I genuinely think it should be required reading for anyone who claims to care about women's health.

My name is Maria Roberts, and in this episode I'm going to be discussing how mainstream feminist reproductive healthcare advocacy, like that from organizations like Planned Parenthood, and these movements built around the language of "choice" and "bodily autonomy," have ultimately failed Black women experiencing maternal mortality. And on top of that, has actively made the political conditions worse. This is a framework which cannot be reformed. The real,

functional alternative, is an intersectional coalition politics built around who the system is already failing. It is truly the only approach that can actually work.

SECTION 1: What mainstream feminist healthcare advocacy actually does

I'll be specific. What I mean when I say mainstream feminist reproductive healthcare advocacy is not talking about feminism broadly. I'm talking about the well-funded and most politically visible strand of feminist healthcare work in this country. This specific strand has spent decades organizing almost exclusively around abortion access, framing the issue as a matter of individual bodily autonomy, of "choice."

In "The Trouble with White Women," Kyla Schuller unpacks the internal logic of this strand very clearly. She argues that white feminism, which she does not define by the race of its practitioners but alternatively, its political architecture, has been insanely consistent for nearly two hundred years. It fights for women to gain access to the same positions of power that wealthy white men hold. It preaches "gender equality", but only in terms of individual advancement within existing structures. Most importantly, it treats the protection of reproductive freedom as basically synonymous with protecting abortion access, defining that protection, as Schuller writes, "solely as the ability to prevent and terminate pregnancy" (Schuller 4).

Think about what that definition excludes. The right to survive pregnancy. The right to be believed when you describe your pain. The right to give birth and come home. For Black women, these are central issues.

Black women in the United States die in childbirth at rates comparable to women in colonized nations. The CDC has documented that Black women are 243 percent more likely to die from pregnancy or childbirth related causes than white women. Serena Williams, one of the most

famous and most *resourced* athletes alive, had to fight to get a nurse to take her symptoms seriously after giving birth. Tressie McMillan Cottom held her dead daughter in a hospital hallway.

These are not the women that these white, mainstream feminist campaigns are designed around. Mainstream feminist reproductive advocacy centers a very different body. One whose primary vulnerability is an unwanted pregnancy. I'd like to be clear when I say that this is anything but accidental. Schuller is very explicit about this. You cannot change the structural DNA of white feminism by simply expanding its tent. Making it more inclusive, as she puts it, "only results in longer tentacles wrapped around more necks" (Schuller 12). More diverse spokespeople delivering the same framework is not the same thing as a different framework.

SECTION 2: Why the framework fails, and what Piercy shows us

Building off of this, Cottom explains why individual credentials and awareness campaigns cannot fix this. She develops what she calls "the political economy of incompetence". Healthcare operates through assumptions about who is a competent subject: whose pain is real and whose body the system was meant to serve. These are *structural* biases. For example, when Cottom was not heard as competent as she described her pain to that nurse. The nurse might not have been consciously racist, but that is not irrelevant when the system had already decided what a competent patient looks like. Her education, her insurance, nothing she possesses could override the perception of her Blackness when she walks into a room. She writes that when medicine "systematically denies the existence of Black women's pain," it marks them as "incompetent bureaucratic subjects" and then serves them accordingly (Cottom 2).

I'm gonna take a little segway and more on Marge Piercy's novel "Woman on the Edge of Time", because, even though it is fiction, it's on-theme. In short, the novel follows Connie Ramos who is a Chicana and Puerto Rican woman navigating poverty and psychiatric institutionalization. At one point we learn that Connie has had a nonconscious hysterectomy performed on her. She is treated with such little regard. "Unnecessarily they had done a complete hysterectomy because the residents wanted practice" (Piercy 43). Connie lost her reproductive capacity; all because her body was treated as a training tool. She was poor, Latina, and institutionalized, and because of this, the system had already decided she was not a competent subject worth protecting.

This is actually the same political economy Cottom describes, the same one that mainstream feminist "choice" rhetoric cannot address, because Connie never had a choice to begin with. The framework of bodily autonomy only protects you if the system already recognizes your body as worth protecting.

SECTION 3: What actually works

In "Punks, Bulldaggers, and Welfare Queens," Cathy Cohen argues that movements built around a single identity axis end up serving the most privileged members of their constituency while abandoning the most marginalized. Her alternative is intersectional coalition politics, a movement built around your shared relationship to a system that is already failing you.

She uses ACT UP's needle exchange and prison projects in the 1990s as a model. People from completely different backgrounds came together because the state was letting them die and they all knew it. It's also documented that nearly 80 percent of women prosecuted for drug use during pregnancy are women of color. The same state that ignores Black women in labor is the same state prosecuting them for their pregnancies. THESE ARE NOT SEPARATE ISSUES.

"Woman on the Edge of Time" follows this in a similar realm when Connie's utopian future, Mattapoissett, tries to resolve inequality by abolishing racial categories entirely. However, the expert notes that readers of color tend to experience that as erasure instead of liberation. And as a Black woman, I completely agree. You cannot build a genuinely inclusive future by acting as though the categories that formed people's oppression are nonexistent. Coalition politics simply asks that you recognize what structures you share with others who are also being failed. You do not need to dissolve your entire identity within it. This is a very important clarification I feel I must make.

CONCLUSION

Cottom writes that "once these gears are in motion, you can never be competent enough to save your own life" (Cottom 4).

Connie Ramos lost her womb in a hospital hallway because residents needed practice. Tressie McMillan Cottom lost her daughter in one because nobody listened. This is the result of a system organized around the disposability of certain bodies, individual actors mustn't bear the full blame.

The politics that can address this don't begin with gender as a universal category, we know that much for sure. We have to start with the most marginalized and work outward from there. That's just not how mainstream feminism tends to operate, so I'll leave you all with this question: are people with platforms willing to follow the lead of those they've failed time and time again?

Works Cited

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